CMS-0042-NC Comment Letter

Submitted by: Kyruus Health | CAGE: 9F6W1 | UEI: MCX5H613G515
Re: Request for Information – Health Technology Ecosystem (CMS-0042-NC)
Date: 16 June 2025
To: Stephanie Carlton, Thomas Keane, MD, and Steven Posnack

Thank you for the opportunity to respond to your Health Technology Ecosystem Request For Information (RFI). Kyruus Health is the leading care access platform helping connect people to the right care. Our solutions facilitate over 300M annual searches for care across our customer base of over 1,400 hospitals, 500 medical groups, and 100 health plan brands. Collectively this represents the voice of roughly 500,000 providers and 75M health plan members. We also power the TriWest provider directory for TRICARE beneficiaries in the West Region and Wellhive's External Provider Scheduling (EPS) search and schedule capabilities for Veterans Affairs (VA). With our large payer and health plan client base, we are a leading independent provider of cost transparency data, serving roughly 75 million health plan members daily. We understand the complexity of these important types of data in the vital care decisions people make every day across the country.

Supporting this broad and diverse group of constituents across the healthcare ecosystem has given us great insight into how our industry can evolve to enhance digital experience and data access, reduce provider administrative burden, and cultivate an interoperable health technology ecosystem that drives real-world outcomes. We are pleased to offer perspectives from our experience on questions primarily from section E. Technology Vendors, Data Providers, and Networks (TD).

Introduction

Kyruus Health is laser-focused on creating neutral and robust national provider directories that engage beneficiaries, spur price transparency, and advance ecosystem interoperability. Data sharing and standardization evolutions have positively impacted our healthcare system, however to truly make an interoperable ecosystem for the future we must focus on making accurate, relevant information freely and securely available in workflows for every stakeholder. Looking first at **digital experience goals** for all stakeholders, we can better understand and codify the **interoperability foundations** and the **workflow infrastructure** needed to support those experiences.

The Goal: optimized digital experiences for all stakeholders

Many of the care access challenges are well understood and documented - extensive wait times to see providers, lack of transparency in terms of availability for new patients, inability to digitally transact in a modern/consumer-oriented fashion, continued opacity in terms of economic burden, and inconsistency of experiences depending on the consumer's origin as a patient or a member. Behind these challenges lie two common themes: (1) our industry has prioritized billing and administrative data over data one needs when seeking care, and (2) our industry has defined success based upon the presence of data instead of the ability to to act upon that data appropriately and in context for the benefit of the patient and the care provider.

The Foundation: accurate, relevant, and complete data

We share your goal for patients having access to information at their fingertips and assistance in navigating healthcare challenges at the most opportune times in their care journey. Our experience has proven that to improve the digital patient experience and access to care, the government must think holistically about what it is that people need when seeking care. For example, NPPES data has the "basics" of data elements that are clearly important. However, we know from our aforementioned 300M searches per year that patients and members require a much more robust data set to meet their needs and compel action. They need everything from clinical elements (specialty, conditions, symptoms, treatments, and procedures), enhanced provider demographics (such as languages spoken and accurate locations), data to support personal preferences and provider match (professional statements, videos, ratings, reviews), location (proximity to home or work), cost, and appointment availability and actionable scheduling capabilities. (TD-2)

Today, much of this vital information either doesn't exist, is inaccurate, or sits behind an economic or technical tollgate that prevents innovators from unleashing the data to the patient or member seeking care. Independent, interoperable, and robust provider directories will be an accurate and easily accessible source of such data, improving healthcare access and operational efficiency across the entire ecosystem. Provider organizations will be matched with the patients that best meet the qualifications of their organization and clinicians. Payer organizations will reduce member friction and lower the operating costs of helping their membership find care. Most importantly, beneficiaries will have unfettered, non-comercialized access to the data they need to plan and coordinate their care. Given the nature of these benefits, who bears the cost is naturally a more difficult question. However, the government must avoid creating information blocking scenarios that squelch innovation and information access by beneficiaries and their care providers. (TD-5; PC-2c)

CASE STUDY: Transparency in Price

An excellent example of establishing and maintaining a complex data foundation is our work in support of Transparency in Coverage legislation. We serve more than 75M members across dozens of health plans nationally, so we know the challenges to compliance innovation. They range from requiring large volumes of irrelevant data that is posted in machine readable files (i.e. behavioral health providers being listed for knee replacement billing codes), the architecture around service level pricing (consumers want to understand the cost of an encounter, not of a single service), and a lack of benefit information or benefit programs that members may be eligible to receive. Collectively, this makes building solutions more complex/expensive while simultaneously leaving out critical components to estimate the patient's cost of care. More targeted cost estimation APIs similar to the Real-Time Prescription Benefit (RTPB) would motivate solution development by allowing new types of consumer facing cost transparency solutions to be more efficiently built and better meet the needs of patients/members. Kyruus Health is actively engaged in this area. We collaborate with Da Vinci on the Patient Cost Transparency (PCT) pilot to prove out a FHIR based, interoperability solution of real time cost estimation for an entire encounter. This could empower innovators to leverage tailored and easier to consume APIs to build out more robust cost transparency with a lower lift empowering patients with true transparency. (TD-19)

The Infrastructure: workflow based data access and interoperability

Increasing access to accurate and relevant CMS data will stimulate developer interest and drive patient and member adoption. Standards like FHIR and USCDI are very helpful and can be foundational for future growth. Our experience sourcing data from myriad systems across the current health information landscape, though, has proven that one size cannot, and should not, fit all. We appreciate the increased focus on APIs as we believe integration of data in the future must be workflow based, not system centric (e.g. the problem is not only the data itself but rather ensuring that the right data is available to the end patient/member/operator when making a decision). Standardized data that is not shared or used in the context of critical workflows will not advance innovation. The use of APIs allows applications to consume relevant and accurate data "in the moment" to help beneficiaries or operators make the right decisions. Increased API-enablement focus would shift value from the presence of data in a system towards what a system or user can do with the data. (TD-9)

The greatest challenge today is the lack of industry support for this approach. There are the obvious challenges of lack of real-time/near real-time data updates (e.g. if a provider accepts new medicare patients, offers virtual care, etc.), expensive integration and/or API fees from EMR companies, and varying data/file formats and disparate data silos. The common thread for all stakeholders is the urgent need to engage with people at the most critical points along their journey to care. Applying today's technology to empower Medicare beneficiaries as they seek, access, and pay for care offers great opportunities to improve access, lower costs, and increase satisfaction for beneficiaries and care givers alike. We believe additional APIs such as a "National Provider and Location directory API" with things like appointment availability can help alleviate these challenges and put us on a path to exposing this information wherever the patient/member begins their journey. (TD-2-a; TD-2-c)

CASE STUDY: FHIR based Provider Directory APIs

Our work leveraging the FHIR Provider Directory API using the Da Vinci PDex Plan-Net Implementation Guide shows how workflow based infrastructure can be built on an open data foundation. We have actively deployed FHIR-enabled solutions to our customer base including the ability to ingest provider data as well as the ability to FHIR enable their provider directory such that third parties can access the information they need via APIs. This example of data standards in action shows great promise for scaling the volume and variety of data needed to support all users engaging with national provider data.

CMS can encourage the expansion of this model by highlighting organizations that actively use and expose open APIs using FHIR and other CMS protocols. When vendors submit data under CMS programs, you could require them to indicate if APIs are used and if they are standards-based or proprietary (visibility vs. enforcement). This approach is a means of acknowledging the federated nature of healthcare data, is more efficient than traditional query methodologies, and enables developers to surface accurate data "in the moment" for providers, beneficiaries, and payers. (TD-4; TD-14)

Conclusion: Driving innovation in digital health

Kyruus Health is dedicated to fostering a healthcare ecosystem that champions optimized digital experiences, ensures access to accurate and complete data, and builds workflow-based interoperability. To achieve this, we specifically recommend that CMS prioritizes the development of neutral and robust national provider directories with comprehensive patient-centric data (TD-2), encourages targeted cost estimation APIs similar to RTPB (TD-19), and supports the creation of a National Provider and Location directory API that includes appointment availability (TD-2-a, TD-2-c). Furthermore, we urge CMS to actively avoid information blocking scenarios (TD-5; PC-2c) and to incentivize the widespread adoption and exposure of open, standards-based APIs like FHIR, potentially by requiring vendors to disclose their API usage and standards in CMS programs (TD-4).

We fully support CMS and ASTP/ONC's efforts to drive innovation and find new ways forward in partnership with every stakeholder of the healthcare ecosystem. We believe that by focusing on these critical areas, in partnership with CMS, we can significantly enhance care access, reduce administrative burdens, and drive meaningful outcomes for all stakeholders across the healthcare landscape.

Thank you for the opportunity to offer our perspectives, comments, and insights, and we eagerly anticipate continuing this crucial dialogue.

Sincerely,

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Paul Merrild, President

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